

A SOUTH AFRICAN BRAND

# ORIGIN

BY ILCO FARMING · REPUBLIC OF SOUTH AFRICA

FRANCHISE PROSPECTUS · 2026

## OWN A STAKE IN SOUTH AFRICA'S MOST REGULATED CANNABIS BRAND

— *TURNKEY, GOLD-STANDARD, DOCTOR-LED.*

A nation-wide network of **Origin online stores** serving patients across South Africa from a single regulated farm — built on a four-licence regulatory backbone, a live AI compliance engine, and a hash-chained audit trail SAHPRA can replay on demand.

9

CITIES · PHASE 1

100

SITES · PHASE 3

45 / 45 / 5 / 5

NET-RETAIL REVENUE SPLIT

## 01 THE SOUTH AFRICAN MEDICAL CANNABIS OPPORTUNITY

A NEW CATEGORY · A REGULATED MARKET · FIRST MOVERS  
WIN THE GATED ESTATES

South Africa has a four-licence regulatory stack for medical cannabis — **SAHPRA Section 22C, SAPC Y-number, HPCSA prescriber, Section 21 access** (*subject to medicines-law specialist sign-off before any final shipped copy*) — that allows a vertically-integrated brand to sell medical-grade product through doctor-led channels. The brands that are *regulated, branded and doctor-led* on day one are best positioned for the medium-term opportunity.

### PHASE 1 FOOTPRINT

9

Online stores, one per major SA metro

### PHASE 3 TARGET

100

Online stores nationwide ·  
scaled to current + grown farm capacity

### CHANNEL

SECTION 21

Doctor-led prescribing route  
(verification pending)

### ORIGIN RUNWAY

READY

Platform live · brand registered ·  
first-tenant operational

**Market-data note.** Population, estate counts, market size and CAGR figures have been deliberately omitted from this draft pending verified sources (SAHPRA annual report, New World Wealth gated-estate counts, dispensary price audits). They will be added once cited.

## 02 THE ORIGIN PROMISE · WHAT THE BRAND STANDS FOR

FOUR GUARANTEES ON EVERY GRAM



### COMPLIANCE-FIRST

Every batch tracked from seed to script under SAHPRA Section 22C. Every action hash-chained. SAHPRA inspectors can replay any decision on demand.



### SA-GROWN · ESTATE-GRADE

Cultivated in South Africa under GACP, processed under GMP, packaged for the world's most demanding patient channel: gated-estate medical cannabis.



### DOCTOR-LED PRESCRIPTION

Section 21 access through HPCSA-registered prescribers. Origin trains doctors on the R 3,500 cannabinoid course; they prescribe, we dispense, the patient gets care.



### TURNKEY FOR THE FRANCHISEE

Brand · platform · SOPs · supply · doctor network · compliance backbone. You bring the location and the local relationships. We bring the rest.

## 03 THE FRANCHISE MODEL · NUMBERS UP FRONT

NO SURPRISES · NO PLATFORM FEES ON TOP · ZAR EVERYWHERE

ONCE-OFF FEE

**R 50,000**

**Deferred** until your first 500g of qualifying sales

SAAS THEREAFTER

**R 1,850 /mo**

Full Origin platform, AI assistants and compliance backbone

DOCTOR COURSE

**R 3,500**

One-off cannabinoid course for your prescribing doctor

FRANCHISEE SHARE

**45%**

Of net retail · per the 45 / 45 / 5 / 5 split (full breakdown overleaf)

### YOUR JOB · SIGN UP PATIENTS

The online-store franchisee is a **patient acquirer**, not a shopkeeper. Your day is spent finding patients in your catchment — gated estates, doctor referrals, social, word-of-mouth — walking them through the Section 21 onboarding flow, and managing their ongoing prescriptions. The platform handles the rest: dispensing, compliance, audit, supply, the lot. **Every patient you sign up is recurring revenue.**

### WHY THE DEFERRED ENTRY FEE?

We win when you win. The R 50,000 once-off only becomes payable after your store moves its first **500 grams of qualifying medical-grade product**. At an ILCO-confirmed average of **60 g per patient per month**, that's **about 9 active patients** — once you cross that threshold, the deferred fee triggers. We don't get paid until the model has proven itself in your hands.

### WHAT R 1,850 / MONTH ACTUALLY BUYS

A live, turnkey Origin store: branded online storefront, patient onboarding wizard, prescription queue, dispensary tracker, AI concierge, Drift Daemon, audit chain, the 76 ILCO SOPs embedded as tap-to-fire workflows. Less than the cost of one part-time consultant — for the entire compliance and operational backbone.

### WHY R 3,500 FOR THE DOCTOR COURSE?

Section 21 prescribing requires an HPCSA-registered doctor familiar with the SAHPRA process and cannabinoid pharmacology. The R 3,500 course is HPCSA-CPD-aligned, takes a day, and qualifies your prescribing doctor to issue Section 21 scripts the same week. **One trained doctor unlocks a whole community of patients.**

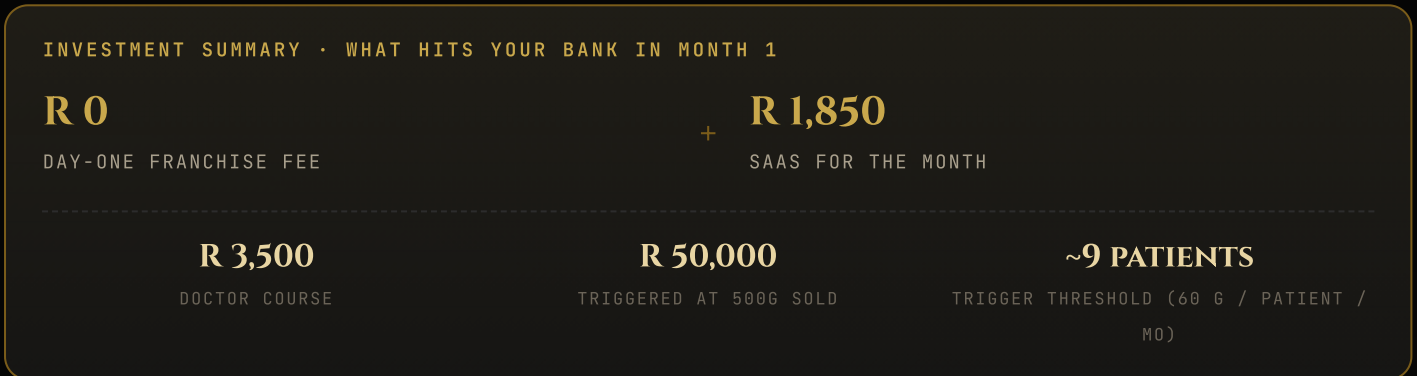
04 **FIVE STEPS FROM APPLICATION TO FIRST  
PRESCRIPTION**

ONBOARDING TIMELINE • TBC PENDING OWNER  
SIGN-OFF



**WHAT YOU DO • WHAT WE DO**

**You bring:** the patient catchment, a prescribing doctor, the local relationships and the marketing legwork to sign patients up. No physical store, no rent, no walk-in retail — Origin runs entirely online. **We bring:** the brand, the regulated supply, the platform, the AI assistants, the SOPs, the audit infrastructure, the marketing collateral, and ongoing compliance support.



05 **REVENUE SPLIT • 45 / 45 / 5 / 5**

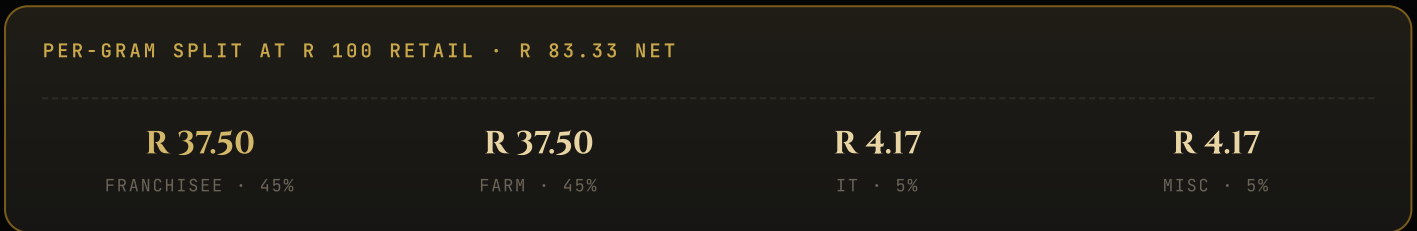
OF NET RETAIL • BENCHMARK R 100 / G (GREEN DOOR)



|                                                                                          |                                                                                               |                                                                                                 |                                                                                                          |
|------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------|
| <b>45%</b><br><b>Franchisee</b><br>Patient acquisition, local team,<br>marketing, profit | <b>45%</b><br><b>ILCO Farm</b><br>Cultivation, processing, GMP<br>packaging, regulated supply | <b>5%</b><br><b>IT &amp; AI Platform</b><br>Origin platform, AI assistants, audit<br>chain, R&D | <b>5%</b><br><b>Misc &amp; Distribution</b><br>Last-mile distribution, network<br>marketing, contingency |
|------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------|

**PER-GRAM RECONCILIATION AGAINST THE R 100 RETAIL BENCHMARK**

Green Door — South Africa's largest medical-cannabis dispensary — retails medical-grade flower at the **R 100 / g benchmark**. After VAT, net retail per gram is approximately **R 83.33**. Applying the 45 / 45 / 5 / 5 split:



**PATIENT ACQUISITION IS YOURS • THE PLATFORM IS OURS**

The R 37.50 / g to the franchisee funds the one job we ask you to own: **signing up patients and keeping them prescribed**. Every other variable — supply, cultivation, GMP, audit, AI, the SOP backbone — is on the platform side. Aligned incentives, not zero-sum: the more patients you sign up, the more we both earn.

**WHAT "NET RETAIL" MEANS**

Net retail = gross retail revenue minus VAT and any direct discount applied at point of sale. The R 1,850 / month SaaS and the once-off R 3,500 doctor course sit *outside* the per-gram split. **No further fees, no advertising tax, no hidden levies.**

06 WORKED EXAMPLE · SCALING BY  
PATIENT COUNT

60 g / PATIENT / MONTH · R 100 / g RETAIL · 45/45/5/5 OF NET  
RETAIL · ALL OWNER-CONFIRMED

The model is fully linear: **each active patient ≈ 60 g / month at R 100 / g**. Below are three scenarios — at the trigger threshold, at modest scale, and at mature scale. Same maths, different patient counts.

| LINE ITEM                              | TRIGGER · 9 PATIENTS | STEADY · 20 PATIENTS | HEALTHY · 30 PATIENTS |
|----------------------------------------|----------------------|----------------------|-----------------------|
| Active patients                        | 9                    | 20                   | 30                    |
| Monthly volume (× 60 g)                | 540 g                | 1,200 g              | 1,800 g               |
| Gross retail (× R 100)                 | R 54,000             | R 120,000            | R 180,000             |
| Net retail (gross ÷ 1.20 · split base) | R 45,000             | R 100,000            | R 150,000             |
| Franchisee · 45%                       | R 20,250             | R 45,000             | R 67,500              |
| Farm · 45%                             | R 20,250             | R 45,000             | R 67,500              |
| IT · 5%                                | R 2,250              | R 5,000              | R 7,500               |
| Misc · 5%                              | R 2,250              | R 5,000              | R 7,500               |
| Less monthly SaaS                      | -R 1,850             | -R 1,850             | -R 1,850              |
| FRANCHISEE · MONTHLY NET               | R 18,400             | R 43,150             | R 65,650              |
| Annualised · × 12                      | R 220,800            | R 517,800            | R 787,800             |

READING THE TABLE

**9 patients** is the deferred-fee trigger threshold (just past 500 g). **20 patients** represents a steady single-store run-rate. **30 patients** represents a healthy single-store run-rate. The model scales linearly — every additional patient adds **~R 2,250 / month** to the franchisee's gross before SaaS. Real per-store ceilings depend on patient sign-up rate and SA market data still pending verification.

**Inputs.** Per-patient consumption (60 g / mo), retail price (R 100 / g · Green Door benchmark) and revenue split (45 / 45 / 5 / 5 of net retail) all owner-confirmed. Net-retail base assumes a 16.67% deduction off gross (≈ VAT) to land on the R 37.50 / gram figure cited by the franchisor. Time-to-trigger and patient sign-up rates are franchisee-dependent and not modelled.

07 THE NATIONAL ROLLOUT · THREE PHASES

FROM CITY FOOTPRINT TO NATION-WIDE PARTNER NETWORK

|                          |                                                                                                                                                                                                                                                                                                                            |                               |
|--------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------|
| PHASE<br>1<br>9 METROS   | <b>NATIONAL ONLINE FOOTPRINT · 9 CITIES</b><br><br>One online Origin store per major SA metro: Johannesburg, Pretoria, Cape Town, Durban, Port Elizabeth, Bloemfontein, East London, Polokwane, Nelspruit. Each store ships to its catchment area; one prescribing doctor per store; the brand becomes nationally visible. | <b>2026</b><br>YEAR 1         |
| PHASE<br>2<br>50 ESTATES | <b>GATED-ESTATE DENSITY · THE 1% OF 1%</b><br><br>Origin online stores targeting the patient catchment inside South Africa's top-tier gated estates. Same online model — what changes is the demographic focus and marketing channel. Estate count and household-level penetration figures pending verified sources.       | <b>2027</b><br>YEAR 2         |
| PHASE<br>3<br>100 SITES  | <b>NATION-WIDE PARTNER NETWORK</b><br><br>Origin scales to <b>100 partner online stores</b> nationwide — paced to grown farm capacity, not aspirational headcount. Every partner runs the same regulated stack, the same audit chain, the same brand. SA's first true national online medical-cannabis franchise.          | <b>2028+</b><br>YEAR 3 ONWARD |

FARM CAPACITY · THE RATE-LIMITER ON ROLLOUT

The rollout is paced to **real ILCO farm capacity**, not aspirational franchisee headcount. Below is what the supply side actually supports today, and how it must grow to fund Phase 2 and Phase 3.

| LAYER                                          | TODAY             | PHASE 3 (REQ'D)                    |
|------------------------------------------------|-------------------|------------------------------------|
| Farm output                                    | 180 kg / mo       | to grow with rollout               |
| % allocated to online channel                  | 25%               | 25%                                |
| Online supply available                        | 45 kg / mo        | depends on grown output            |
| Active sites in network                        | Phase 1 · 9       | Phase 3 · 100                      |
| PER-SITE SUPPLY AT CURRENT CAPACITY            | ~5 kg / site / mo | 450 g if no grow · grows with farm |
| Patients supportable per site (60 g / patient) | ~83 / site        | scales with farm grow-plan         |

**Reading the table.** Today, with 180 kg / mo farm output and 25% allocated to online, each Phase 1 store has roughly **5 kg / month of supply** — enough for ~83 patients (well above the trigger threshold). For Phase 3 to put 100 sites on a similar footing, ILCO farm capacity must grow alongside the rollout. The franchise plan is paced to that grow-plan, not the other way around.

WHY GATED ESTATES FIRST

Top-tier SA gated estates are dense, affluent patient catchments — underserved by current cannabis access channels and ideal for online prescription delivery. They concentrate high-value patient sign-ups in geographies the franchisee can market to efficiently. *(Estate counts pending verified sources.)*

08 WHAT R 1,850 / MONTH ACTUALLY BUYS YOU

A TURNKEY MEDICAL-CANNABIS FRANCHISE, NOT A SOFTWARE LICENCE

- |                                                                                                                                            |                                                                                                                              |
|--------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------|
| ✓ <b>Origin brand &amp; collateral</b> — registered trade marks, packaging, signage, marketing assets, social-media kit.                   | ✓ <b>76 SAHPRA-aligned SOPs</b> — every standard operating procedure, embedded as tap-to-fire workflows in the platform.     |
| ✓ <b>TnT-ZA platform access</b> — full track-and-trace from seed to script, per-role dashboards, ticket system, real-time inventory.       | ✓ <b>Five AI assistants</b> — Concierge, SMF Composer, Ops Q&A, Tickets, Drift Daemon. Always on, always grounded.           |
| ✓ <b>Doctor network &amp; course</b> — referral pipeline of HPCSA prescribers and the R 3,500 cannabinoid course for your in-house doctor. | ✓ <b>Compliance backbone</b> — hash-chained audit log, sign-off chain (RP → DAR → AR), Site Master File live and signed.     |
| ✓ <b>Regulated supply from ILCO farm</b> — GACP-cultivated, GMP-processed, fully traceable, on-demand replenishment.                       | ✓ <b>SAHPRA + SAPC paperwork</b> — Section 21 packet generator, S6 register, adverse-drug-reaction workflow, all auto-filed. |
| ✓ <b>Onboarding &amp; training</b> — staff training matrix, role-based dashboards, new-hire wizard, refresher reminders.                   | ✓ <b>24 / 7 support</b> — platform uptime monitoring, compliance hotline, escalation to ILCO regulatory team.                |

WHAT THIS REPLACES

A team of 4–6 specialists you would otherwise need to hire: a regulatory consultant, a quality manager, a software engineer, a brand director, a doctor liaison, and a compliance officer. Origin gives you all six functions, on platform, on day one — for less than the cost of one part-time consultant.



09 FOUR WAYS TO PARTICIPATE · CHOOSE  
YOUR PATH

INDICATIVE · SUBJECT TO QUALIFICATION & ILCO BOARD  
APPROVAL

PATH A · SINGLE ONLINE STORE

One Origin online store in one metro. Lowest entry, fastest payback, ideal for owner-operators who can drive patient sign-up.

**R 50K**

ONCE-OFF ·  
DEFERRED

**R 1,850**

SAAS / MO

**45%**

OF NET RETAIL

PATH B · MULTI-STORE METRO

Multiple Origin stores across a single metro under one operator. Bundled SaaS & co-marketing terms by negotiation.

**BY NEG.**

BUNDLED ENTRY

**BY NEG.**

SAAS BUNDLE

**45%**

OF NET RETAIL

PATH C · GATED-ESTATE CLUSTER

Origin online store with a premium estate as its primary patient catchment. Marketing focused on the estate community; doctor partnerships drive sign-up. Same online economics, premium-density target.

**BY NEG.**

ONCE-OFF ·  
DEFERRED

**BY NEG.**

SAAS / MO

**45%**

OF NET RETAIL

PATH D · EQUITY IN ILCO FARMING

Direct equity in the parent. Exposure to the farm, the platform, and the partner network. By invitation only.

**BY NEG.**

MIN TICKET

—

BOARD-APPROVED

**EQUITY**

IN PARENT CO.

▲ YEAR-ONE RETURN PROJECTIONS WITHHELD PENDING VERIFIED MARKET DATA

Per-path year-one net returns are deliberately not quoted in this draft. They depend on patient sign-up rates, monthly consumption, and competitive pricing — three figures that need verified SA-market sources before they can be put in front of investors. The unit economics on page 06 hold; the per-store run-rate that drives a year-one number does not yet have a defensible source.

10 **WHY THIS WORKS · THE FIVE MOATS**

WHAT COMPETITORS CANNOT COPY IN UNDER 24 MONTHS

**1 · REGULATORY STACK**

Four South African licences (SAHPRA Section 22C, SAPC Y-number, HPCSA prescriber, Section 21 access) wired together at the platform layer. Every gram is sold under all four. Imitators need 12–18 months and a regulatory consultant to even attempt this.

**2 · THE TECHNOLOGY PLATFORM**

TnT-ZA: track & trace, world-model state, hash-chained AuditLog, five purpose-built AI assistants (Opus 4.7 + Sonnet 4.6 + Haiku 4.5), Drift Daemon, ghost mode, sign-off chain. Built over 18 months on top of a regulated cultivation operation.

**3 · THE BRAND**

Origin™ is registered, premium, gold-on-black, and aimed at the most discerning patient channel. Doctor-led, estate-grade, SA-grown. Brand equity compounds with every store launched.

**4 · THE DOCTOR NETWORK**

Origin trains and onboards prescribing doctors directly. Every franchisee gets access to the network from day one. As Section 21 prescribing scales, the network compounds with it.

**5 · THE SUPPLY CHAIN**

ILCO Farming cultivates, processes, GMP-packages and ships nationally — without a third-party in the chain. Every batch carries a hash-chained provenance trail back to the seed. No competitor in SA can match this end-to-end.

**THE COMPOUNDING EFFECT**

Each moat reinforces the others. The regulatory stack only works at scale because the platform automates the paperwork. The brand only premiums because the supply is regulated. The doctor network only grows because the brand is trusted. **This is how a category leader is built.**

## TAKE THE FIRST STEP.

If the model fits, here's how to move from interested to onboarded.

### STEP 1

#### EXPRESS INTEREST

Email or call. We arrange a 30-minute conversation to understand your location, your community and your goals.

### STEP 2

#### QUALIFY & DEMO

We invite you to a live platform demo and a visit to the ILCO farm. You meet the team, see the cultivation, and walk through a live online store on the platform.

### STEP 3

#### SIGN & LAUNCH

Franchise agreement signed, platform provisioned, doctor course booked. First scripts inside 6 weeks.

#### Origin Franchise Office · ILCO Farming (Pty) Ltd

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**ORIGIN™**

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